

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 74 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: July 26, 2019 | Provider: Thomas Reed, MD — Pulmonology | Doc ID: RFH-2019-0726-PULM

Pulmonology Consultation

Chronic Dyspnea & Cough — COPD Diagnosis

Reason for Consultation

Chronic productive cough and exertional dyspnea; incidental emphysema on PET/CT (03/2019); 40 pack-year ex-smoker (quit 2012).

HPI

74-year-old reports morning cough with whitish sputum and breathlessness on exertion, partly overlapping with heart failure symptoms but persisting despite euvolemia. Two 'chest cold' episodes last winter requiring antibiotics. No hemoptysis.

Pulmonary Function Tests (07/22/2019) — Results

Post-bronchodilator FEV1/FVC 0.61 (obstructive). FEV1 1.62 L = 58% predicted. FVC 2.66 L = 78% predicted. Minimal bronchodilator reversibility (+6%). DLCO mildly reduced (62% predicted). Consistent with moderate COPD (GOLD grade 2, group B).

Assessment

- 1. Chronic obstructive pulmonary disease, moderate (GOLD 2), tobacco-related — distinct from but co-existing with HFrEF.
- 2. Overlapping dyspnea from HFrEF — ensure euvolemia.
- 3. Emphysema on imaging.

Plan

- Start tiotropium (LAMA) 18 mcg inhaled once daily.
- Add an as-needed short-acting bronchodilator (albuterol) inhaler.
- Pneumococcal (PCV) and annual influenza vaccination — ensure up to date.
- Pulmonary rehabilitation referral; reinforce continued abstinence from tobacco.
- Low-dose CT lung cancer screening eligible (>20 pack-years, age) — ordered.
- Follow-up 3 months; spirometry annually.

Respiratory Medications

Medication	Dose	Route	Frequency	Indication
Tiotropium (Spiriva)	18 mcg	Inhaled	Once daily	COPD maintenance (LAMA)
Albuterol HFA	90 mcg	Inhaled	PRN (2 puffs q4-6h)	Rescue bronchodilator